bone-density decline," says his physician.

US drug regulator Food and Drug Administration (FDA) labels warn doctors about key red flags before prescribing these drugs: a personal/ family history of thyroid cancer remains a strict contraindication, and patients should be counselled on thyroid-tumour symptoms. In India, while prescriptions require obesity or diabetes criteria, there is no standardised checklist. Baseline evaluations—glycaemic testing (for diabetes), eye examinations in high-risk diabetes, renal function, abdominal imaging—vary widely across clinics.

A physician in Lucknow describes being pressured to prescribe Rybelsus to a politically well-connected patient. Standard investigations—ultrasound of the neck and abdomen, glucose-tolerance test and an eye examination—were not conducted. Months later, the patient developed visual disturbances and thyroid enlargement, complications now being contested in court. "The prescription wasn't the problem," the doctor says. "The absence of screening was." In India's loosely regulated environment, he adds, clinicians are vulnerable—caught between patient demand, institutional pressure and the lack of clear protocols.

And with cheaper generic weight-loss drugs set to be available soon, experts fear an epidemic of indiscriminate pill-popping. Indian regulators recently approved generic versions of semaglutide, as patents expire in March 2026. Soon, Indian firms like Dr Reddy's Laboratories and Sun Pharma are expected to launch their own varieties of drugs like Ozempic and Wegovy at 50-60 per cent lower prices.

THE METABOLIC DRUGS BOOM

AVAILABLE IN INDIA

MOUNJARO
(Eli Lilly)



GLP-1/GIP agonist; approved for diabetes and weight loss; used widely in India since March 2025

COST: Pens priced around Rs 14,000–Rs 27,500/ month



OZEMPIC
(Novo Nordisk)

Semaglutide (GLP-1 receptor agonist) injectable for diabetes and used off-label for weight loss; launched in India in December 2025

COST: Rs 8,800–Rs 11,175/ month for basic doses

WEGOVY
(Novo Nordisk)

Semaglutide approved specifically for weight management; available since mid-2025



COST: Rs 16,400–Rs 26,000/ month depending on strength

RYBELSUS
(Novo Nordisk)

Oral semaglutide for diabetes; widely prescribed since its India entry in 2022; used off-label for weight control

COST: Rs 2,300-3,100 for a strip of 10 tablets



COMING SOON

ORFORGLIPRON
(Eli Lilly): **A fully oral GLP-1 agonist in late trials; will increase access for patients who avoid injections**

RETATRUTIDE
(Eli Lilly): **This 'Triple G' targets GLP-1, GIP and glucagon together; has shown up to 23–24 per cent weight loss, better cardiometabolic benefits**

GENERIC VERSIONS OF SEMAGLUTIDE/ GLP-1 AGONIST DRUGS
(Indian firms) **Regulators have approved generic versions of Semaglutides like Wegovy and Ozempic; prices to fall by 50-60 per cent**

THE OBESITY CRISIS

There is no denying that India is in the throes of a metabolic health crisis. The 2025 ICMR-INDIAB analysis found that 43.3 per cent of Indian adults are Metabolically Obese Non-Obese (MONO)—people with a normal Body Mass Index (ideal weight after measuring it against one's height) but with high waist circumference, elevated blood sugar, hypertension or insulin resistance. Another 28.3 per cent are Metabolically Obese Obese (MOO), carrying the highest risks of Type-2 diabetes (where insulin, the hormone that pushes food broken down into blood sugar/ glucose, into cells to turn it into energy, cannot be absorbed by cells), fatty liver disease, and kidney and heart diseases.

India's unique metabolic profile further complicates decisions. As Dr Anoop Misra, chairman, Fortis C-DOC Centre of Excellence for Diabetes, Metabolic Diseases and Endocrinology, notes, Indians reach metabolic risk at younger ages, with progression from normal glucose to diabetes occurring almost twice as fast as in Western populations.

It is this dire situation that has triggered the boom of metabolic medicines. "Metabolic medicine has